Capivasertib + fulvestrant

Capivasertib is taken twice daily for four days, then stopped for three days, each week. Fulvestrant injections are given on Days 1 and 15 of the first 28-day cycle, then Day 1 of later cycles. Treatment continues while helpful and tolerated.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Planned review points are not equal time intervals or booked tests. Fasting glucose is preferably checked before the dose on day 3 or 4 of a dosing week. The team agrees additional home monitoring.

Check	Before starting	First 2 weeks	Weeks 3–8	After week 8	Pause / changed treatment
Fasting glucose	Baseline; optimise control	Weeks 1 and 2; extra checks by risk	Weeks 4, 6 and 8	Monthly; extra checks by risk	Review on and off tablet days if glucose rises
glycated haemoglobin (longer-term blood glucose) (HbA1c)	Baseline HbA1c	Additional tests if indicated	Week 4 if diabetes, prediabetes or high baseline glucose	Every three months	Individual diabetes review
Blood / kidney / liver	Baseline full blood count (FBC), renal and liver profile	Early symptoms may need extra tests	Before each 28-day cycle	Before each cycle	Review abnormal results before a restart
Skin / injection checks	Review skin and bleeding risks	Report rash and injection symptoms	Check skin, hydration and injection tolerance	Continue symptom and medicine review	Treat and follow unresolved effects

THE WHOLE TREATMENT JOURNEY

Capivasertib + fulvestrant: treatment and checks

Gene-altered, ER-positive / HER2-negative advanced breast cancer

Capivasertib is taken twice daily for four days, then stopped for three days, each week. Fulvestrant injections are given on Days 1 and 15 of the first 28-day cycle, then Day 1 of later cycles. Treatment continues while helpful and tolerated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First 2 weeks	Weeks 3–8	After week 8	Pause / changed treatment
Treatment plan NCCP 00947	Confirm biomarkers and prescription	Start four-on/three-off tablets; fulvestrant Day 1	Extra fulvestrant Day 15 in cycle 1; then Day 29	Continue tablet pattern and 28-day injections	Review each medicine before restart
Fasting glucose NCCP 00947	Baseline; optimise control	Weeks 1 and 2; extra checks by risk	Weeks 4, 6 and 8	Monthly; extra checks by risk	Review on and off tablet days if glucose rises
HbA1c NCCP 00947	Baseline HbA1c	Additional tests if indicated	Week 4 if diabetes, prediabetes or high baseline glucose	Every three months	Individual diabetes review
Blood / kidney / liver NCCP 00947	Baseline FBC, renal and liver profile	Early symptoms may need extra tests	Before each 28-day cycle	Before each cycle	Review abnormal results before a restart
Skin / injection checks NCCP 00947	Review skin and bleeding risks	Report rash and injection symptoms	Check skin, hydration and injection tolerance	Continue symptom and medicine review	Treat and follow unresolved effects

What each check is for

1 Confirm the selected pathway

Before starting

Sources: NCCP 00947

2 Glucose and HbA1c

Baseline; weeks 1, 2, 4, 6 and 8; then ongoing

Sources: NCCP 00947

3 Symptoms and treatment changes

Throughout, including tablet-free days

Sources: NCCP 00947

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.